



Клиники неотложной медицины Северной Америки

Том 25, выпуск 2, май 2007, страницы 249–281

Как вести себя с пациентом, принявшим неизвестное вещество

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Токсическая передозировка может проявляться различными клиническими признаками и симптомами. Они могут быть единственными подсказками для постановки диагноза, если на момент первичного осмотра причина отравления неизвестна. Прогноз и клиническое течение выздоровления пациента, отравившегося определенным веществом, во многом зависят от качества медицинской помощи, оказанной в первые несколько часов после поступления в отделение неотложной помощи. Обычно препарат или токсин можно быстро идентифицировать с помощью тщательного сбора анамнеза, целенаправленного физикального обследования и доступных лабораторных тестов. После стабилизации состояния пациента врач должен решить, как снизить биодоступность еще не всосавшегося токсина, какие антидоты (если таковые имеются) следует ввести и нужны ли другие меры для ускорения выведения токсина из организма.

Доступ через вашу организацию

Проверьте доступ к полному тексту, войдя в систему через свою организацию.

Доступ через **вашу организацию**

Фрагменты разделов

Клинические рекомендации

Несмотря на то, что существует несколько опубликованных заявлений о позиции [2], [3], [4], [5], [6] и практических рекомендаций или консенсусных заявлений [7] по вопросам клинической токсикологии, диагностики и лечения, большая часть литературы основана на ретроспективном анализе серий клинических случаев или отдельных сообщений о случаях, а также на исследованиях на животных или лабораторных исследованиях. Хорошо контролируемые рандомизированные исследования с участием людей и достаточным количеством испытуемых проводятся редко и требуют больших усилий.

Данные Регионального центра по борьбе с отравлениями существуют и регулярно обновляются. ...

Эпидемиология

В 2004 году в Американскую ассоциацию токсикологических центров поступило более 2,4 миллиона сообщений о случаях отравления людей токсичными веществами [8]. Более 75 % случаев произошли в домашних условиях, 15 % — в медицинских учреждениях. Две трети случаев отравления произошли с участием детей в возрасте до 20 лет. Чаще всего люди травились чистящими средствами, затем следовали анальгетики и косметика/средства личной гигиены [8]. Было зарегистрировано 1183 случая смертельного отравления детей в возрасте до 6 лет. ...

Токсикокинетика

Что же тогда не является ядом? Все есть яд, и ничто не лишено ядовитости. Только доза определяет, является ли вещество ядом.

— Парацельс (1493–1541), отец токсикологии эпохи Возрождения, в своей *Третьей защите*[11] ...

По мнению токсиколога эпохи Возрождения Парацельса, любое вещество можно считать потенциальным ядом в зависимости от дозы и продолжительности воздействия. Фармакокинетику ксенобиотиков в организме человека можно описать следующим образом: ...

Дифференциальная диагностика при отравлении

Any symptomatic patient can be a potential drug overdose. Altered mental status, gastrointestinal complaints, cardiovascular compromise, seizures, and temperature-related disorders can all be toxin-related. Some are subtle, such as the flulike symptoms seen with carbon monoxide poisoning, whereas cardiotoxins, such as digitalis, may mimic intrinsic heart disease. In the differential diagnosis, the clinician should also consider similar agents. For example, the following should be considered ...

Prehospital care

Apart from basic stabilization measures (such as oxygen administration, cardiac monitoring, and venous access establishment), emergency medical system (EMS) personnel need to do little in the field with the overdosed patient, especially when the transport time to the nearest hospital is short. EMS personnel should avoid giving ipecac because of the possibility of delaying definitive therapy and the potential for aspiration in the comatose or combative patient. Some clinical trials have studied ...

The general approach

The general approach to the diagnosis and management of the poisoned patient can be described using a two-pronged model as seen in Fig. 1. In practice, the two prongs occur simultaneously. The left-sided prong begins with basic emergency medical care—the ABCs (airway, breathing, circulation). The mnemonic *DONT* stands for dextrose, oxygen, naloxone, thiamine. In most potentially poisoned patients, a rapid blood glucose measurement should be obtained and any derangements corrected. Supplemental ...

History

Historical facts should include the type of toxin or toxins, time of exposure (acute versus chronic), amount taken, and route of administration (ie, ingestion, intravenous, inhalation). It is also important to understand why the exposure occurred (accidental, suicide attempt, euphoria, therapeutic misadventure) and whether there is history of psychiatric illness or previous suicide attempts. Furthermore, it is important to inquire about all drugs taken, including prescription, over-the-counter ...

Physical examination

In the emergency setting, performing an overly detailed physical examination is a low priority compared with patient stabilization. Even a directed examination can yield

important diagnostic clues, however. Once the patient is stable, a more comprehensive physical examination can reveal additional signs suggesting a specific poison. Additionally, a dynamic change in clinical appearance over time may be a more important clue than findings on a single examination. There are classic presentations ...

Laboratory tests

It is commonplace for many health care providers to order excessive laboratory tests when treating a poisoned patient. This testing often occurs because the offending agent is unknown or the clinician is unfamiliar with the toxin. Consultation with the regional poison center or medical toxicologist may help to narrow the scope of testing. ...

Abdominal films

A plain abdominal radiograph (KUB) can reveal radiopaque pills, drug-filled packets, or other toxic material. Drugs or toxins that are likely to be visible on films can be recalled by the mnemonic COINS as demonstrated in Box 8[45].

In some cases, the vehicle in which the drug is contained, such as an enteric coating or latex, is more radiopaque than the drug itself. For this reason, a KUB may be useful in cases of body packers (drug smugglers). On the other hand, body stuffers who quickly ...

Toxidromes

A collection of symptoms associated with certain classes of poisons is known as a toxic syndrome, or toxidrome. In patients who have unknown overdoses, a toxidrome can assist in making a diagnosis and is also useful for anticipating other symptoms that may occur. Cholinergic, anticholinergic, sympathomimetic, and narcotic agents all have characteristic toxidromes; withdrawal from many addictive agents may also produce distinctive constellations of symptoms [19]. Box 10 lists the common ...

Treatment

The management of any clinically significant poisoning should begin with basic supportive measures. Most poisoned patients do well with supportive care alone. ...

Cutting-edge toxicology

The latest toxicology antidotes include fomepizole for ethylene glycol and methanol poisoning [88], [89], [90], specific immune therapy with purified Fab fragments for

rattlesnake antivenin [91], [92], [93], [94], and high-dose insulin for calcium channel antagonist poisoning [95], [96]

Current controversies include challenging the widely accepted 72-hour oral *n*-acetylcysteine (NAC) treatment course for acetaminophen toxicity. Many are now suggesting a more abbreviated regimen [97]. An ...

Disposition

People who have a potentially serious overdose should be observed for several hours before discharge. If signs or symptoms of intoxication develop during that time, the patient should be admitted for further observation and treatment. Some agents may require a more prolonged observation period. Such agents include sustained-release products and agents with known delayed or prolonged onset of action: calcium channel antagonists, theophylline, lithium, methadone, Lomotil, monoamine oxidase ...

Summary

The management of the poisoned patient who has an unknown exposure can be diagnostically and therapeutically challenging. The history and physical examination, along with a small dose of detective work, can often provide the clues to the appropriate diagnosis. The two-pronged approach as outlined in Fig. 1 provides a framework for evaluating poisoned patients. Consultation with a regional poison center or clinical toxicologist early in the care of poisoned patient can have a profound impact on ...

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